

Standard Operating Procedure

Verification of Personnel Qualifications and Registration

«Hospital Name» — Supports AAC.1.b

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» verifies the qualifications, registration, and licensure of medical, nursing, and paramedical staff before they provide clinical care, and how that verification is re-confirmed on a defined schedule.

2. Scope

This SOP applies to every clinical staff member — doctors, nurses, and paramedical/allied health staff, whether permanent, visiting, contractual, or locum — engaged in any diagnostic or treatment service listed in the hospital's service directory.

3. Responsibility

HR Manager / HR Department: carries out and records primary-source verification for every new hire and every renewal.

Department Head: confirms verification is complete before assigning clinical duties.

Medical Superintendent / Quality Coordinator: audits the verification record at least annually and signs off on the audit.

4. Procedure

4.1 Before a new staff member is assigned clinical duties

1. Collect original or attested copies of: degree/diploma certificate(s), internship/residency completion certificate (where applicable), and the relevant council registration certificate (state Medical Council / Nursing Council / Paramedical Council, as applicable).
2. Verify the registration is current and unrestricted by checking the issuing council's official online registry directly — not by relying on the certificate alone. Take a screenshot or printout of the registry result and file it with the verification record.
3. For doctors, confirm the qualification and specialty shown match the role being offered (e.g., a Cardiology consultant post requires a recognised Cardiology qualification, not a general MBBS alone).
4. Contact the issuing institution directly to confirm the degree/diploma is genuine, wherever the hospital's own risk assessment calls for it (for example: unfamiliar institutions, or any document that looks altered or inconsistent).
5. Record the outcome in the Personnel Qualification & Verification Record (Record 5 of the AAC.1 Required Records pack), including the verification date and the name of the person who verified it.

6. Do not assign independent clinical duties until verification is complete and recorded. Where a role must start before verification finishes, obtain written approval from the Medical Superintendent for time-limited supervised practice only.

4.2 Ongoing / renewal verification

1. Track each staff member's registration expiry or renewal date in the verification record.
2. Re-verify registration status at least once a year for every clinical staff member, and immediately upon renewal of any registration.
3. If a registration has lapsed or been restricted, suspend the staff member's clinical duties immediately and escalate to the Medical Superintendent the same day.

4.3 Visiting, locum, and contractual staff

1. Apply the same verification steps as 4.1 before the first duty, regardless of engagement length.
2. For engagements under [Hospital to define] days, a documented verbal confirmation with the issuing council, followed by written confirmation within 7 days, is acceptable — full original-document collection may follow rather than precede the first shift only where the Medical Superintendent has given prior written approval.

5. Records

Every verification must be logged in the Personnel Qualification & Verification Record. This SOP does not create a separate log — it defines how that record gets filled in correctly.

6. Related Documents

- HCO_AAC_1_v2_REWRITE_DRAFT — AAC.1 policy (this SOP supports AAC.1.b)
- AAC.1 Required Records / Evidence Checklist — Record 5: Personnel Qualification & Verification Record
- Recruitment and credentialing policy (HRM), where a separate hospital-wide credentialing policy exists

7. Review

This SOP is reviewed annually by the Quality Coordinator, or immediately if a verification failure or near-miss reveals a gap in the procedure.

8. References

- The Joint Commission. (n.d.). Credentialing and privileging standards (Standard HR.01.02.05). Primary source verification of medical school diplomas, specialty training/residency certificates, and licenses or registrations required for clinical privileges. <https://www.jointcommission.org/en-us/standards> (Official page; full standard text requires purchase or subscription.)